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Examining the Long-Term Impacts of Psychotropic Drugs and Considerations for People Discontinuing Treatment

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ABSTRACT

Psychotropic drugs dominate the mental healthcare landscape. This is despite contention over their proposed mechanism of action, concerns for their adverse effects, and questionable effectiveness, especially over the long term. Mental health nurses are routinely involved in administering psychotropic drugs, observing for and managing adverse effects, and providing information and support to people prescribed these agents. This critique explores the current understanding of the mechanism of action for psychotropic drugs, evidence for their effectiveness, adverse effect burden and implications for long term use. The role of mental health nurses in deprescribing and supporting people to discontinue treatment is considered.

Background

Psychotropic drugs such as antidepressants, antipsychotics, stimulants and mood stabilisers are prescribed in high numbers and prescription rates are increasing (Brauer et al., 2021). These agents are considered fundamental to the “first line treatment” of most psychiatric conditions and people accessing public mental health services will be offered, expected, and even forced into taking a psychotropic drug.

The justification for the use of psychotropic drugs has traditionally relied on the belief that “mental disorders” have a neuro-biogenic cause or are due to a chemical imbalance of the brain that psychotropic drugs correct. A simple internet search conducted in November 2024 with the question ‘what causes mental illness?’ confirms this view. The Australian Government Department of Health and Aged Care (2024) website (while acknowledging that life experiences, trauma, and abuse contributes to mental illness) states that “biological factors, such as genes, brain chemistry or hormonal changes” are also at play. Similarly, the Mayo Clinic (2024) website affirms that when brain chemicals are impaired, the function of nerve receptors change, leading to depression and other “emotional disorders”. Another authoritative source, the Cleveland Clinic (2024) website declares that mental illness might be the result of abnormal genes and that chemical imbalances in your brain can also lead to mental illness. However, the respected charity Mind (2024) website asserts “there is no

Multiple studies have now cast doubt over any clear or direct neurobiological or genetic cause or association with mental illness (Border et al., 2019; Borsboom et al., 2018; Curtis, 2021; Marsman et al., 2020; Nour et al., 2022; Winter et al., 2024). For example, Nour et al. (2022) surmise that despite three decades of intense neuroimaging research, there is no neurobiological account for any psychiatric condition. Torrey (2024) concludes that not one gene has been found that can be causally linked to ‘schizophrenia’, and despite many studies and variety of techniques used, Parlatini et al. (2024) confirm that no promising biomarkers have been identified for ‘Attention Deficit Hyperactivity Disorder (ADHD)’. The chemical imbalance theory of mental illness and the serotonin hypothesis of depression have been refuted (Ang et al., 2022; Healy, 2015; Hengartner & Plöderl, 2023; Moncrieff et al., 2022, 2023).

While genetics and biology have a role to play in all aspects of the human condition, the degree to which genetics and biology influence mental health and well-being remains a matter of conjecture. What is known without question, is that mental health challenges are predominantly associated with socio-economic disadvantage, as well as trauma, adversity, violence, abuse, neglect, oppression and discrimination (World Health Organization & the United Nations [WHO & UN], 2023).

The fact is, there is no pathophysiology, disease process or one identifiable biomarker associated with any psychiatric diagnosis. They are highly heterogeneous labels

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strong evidence to say that any mental health problems are caused by a chemical imbalance in our brains”.

of questionable scientific validity (Kajanoja & Valtonen, 2024). There are no blood tests or scans. As the chair of the taskforce for the Diagnostic and Statistical Manual (DSM)

for Mental Disorders version four Frances (2013, p. 96) concedes, “mental disorders do not live ‘out there’ waiting to be explained”.

They are constructs that psychiatry has invented “and not very compelling ones at that”.

While there are clearly many people who believe and attest that psychotropic drugs have assisted them, mounting concerns have been expressed over the consequences of long-term use, their adverse effects, the dampening effect they have on emotions and thoughts, compromised functioning and the impact psychotropic drugs have on people’s sense of self and agency (Bjornestad et al., 2020; Cartwright et al., 2016; Dijkstra et al., 2024; Read et al., 2018; Read & Sacia, 2020; Thompson et al., 2020). From a lived experience perspective, Hall (2019) suggests that psychotropic drugs can be desirable for blunting or taking the edge off extreme states. However, they do not address the underlying causes of mental distress and therefore best considered as a coping aid that sometimes alleviates symptoms.

Antidepressants

With antidepressant drug use growing internationally it is disconcerting to have the serotonin hypothesis of depression, which has long been touted to justify their use, now discredited. Of additional concern is the lack of evidence for the effectiveness of antidepressants (Jakobsen et al., 2020; Munkholm et al., 2019) and their enduring adverse effect and withdrawal symptoms. There are numerous critiques of clinical trials and meta-analyses that are cited to justify the prescription of antidepressants. The central criticism is that the evidence for the effectiveness of antidepressants is derived from trials of 6–12 weeks duration when most people take antidepressants for much longer. The effectiveness of antidepressant use over the long term has not been explored. Meta-analyses of short-term clinical trials, mostly on selective serotonin reuptake inhibitors (SSRIs) and selective noradrenaline reuptake inhibitors (SNRIs), have found that the difference between antidepressant and placebo is approximately two points on the clinician-rated Hamilton Depression Rating Scale (HAM-D). However, this difference between antidepressant and placebo does not meet the threshold for clinical significance (Horowitz & Wilcock 2022; Moncrieff et al., 2023; Warren, 2020) and there is evidence that drug-placebo differences might be inflated when efficacy estimates are based on subjective symptom rating-scales such as the HAM-D (Hengartner & Plöderl, 2018). Moreover, there is no evidence that increasing antidepressant dose improves the response, and higher doses have been linked to violence, suicide, homicide, mania, and psychosis (Warren, 2020).

There is increasing recognition of the adverse effect burden of antidepressants which includes somnolence, dry mouth, profuse sweating, weight gain, cardiovascular problems, sexual dysfunction osteoporosis in women, type 2 diabetes, protracted withdrawal and increased suicidality

(Baranova et al., 2025; Cao et al., 2024; Davies et al., 2023; Hengartner et al., 2021; Hengartner & Plöderl, 2019; Horowitz & Wilcock 2022; Lagerberg et al., 2023; Rajha et al., 2025; Wang et al., 2024).

Coupland et al. (2018) conducted a cohort study that involved 238,963 patients aged 20–64 years with a primary diagnosis of depression registered with primary care practices across the UK. Of note, fracture rates increased considerably for SSRIs and other antidepressants compared with periods when antidepressants were not used. All antidepressant classes were associated with substantially increased rates of falls, while mirtazapine was associated with significantly increased mortality rates over 1 and 5 years of follow-up.

Recipients of antidepressants have reported high rates of troubling adverse effects. In an online survey of 1,431 people prescribed antidepressants from 38 countries 71% reported feeling ‘emotionally numb’ and 70% reported ‘feeling foggy or detached’. Respondents also identified with ‘feeling not like myself’ (66%), ‘sexual difficulties’ (66%), ‘drowsiness’ (63%), and ‘reduction in positive feelings’ (60%). ‘Suicidality’ because of the drugs was reported by 50%. Withdrawal effects were reported by 59%, and ‘addiction’ by 40%. One third did not recall being told about adverse effects by the prescriber. Less than 5% were told about suicidality, emotional numbing, withdrawal effects or ‘addiction’ (Read & Williams, 2018).

Antidepressant drug withdrawal was only formally recognised in recent years, traditionally and misleadingly referred to as ‘discontinuation syndrome’ (Davies & Read, 2019). This term was promoted by drug companies to allay patient concerns, however the more pharmacologically accurate term is ‘withdrawal symptoms’, now adopted by the UK Royal College of Psychiatrists, the British Medical Association and national institute for health and care excellence (NICE). Yet there has been limited official acknowledgement of this condition in the USA and elsewhere (Horowitz & Taylor, 2024).

Withdrawal symptoms such as anxiety, insomnia and depression can be mistaken for relapse, however other reactions such as flu-like symptoms, nausea, sensory disturbances, dizziness, electric shock-like sensations, diarrhoea, headaches, muscle spasms and tremors, agitation, sweating and irritability are also commonly reported and are clearly different to the symptoms for which the drug was prescribed in the first place (Davies & Read, 2019; Guy et al., 2020; Horowitz & Wilcock, 2022).

It is also recognised that antidepressant withdrawal can be more severe and long lasting than previously acknowledged, with symptoms persisting for months or more, and can be severe in some instances (Davies & Read, 2019; Guy et al., 2020). Gastaldon et al. (2022) found that antidepressants are associated with a disproportionate reporting of withdrawal, with reporting differences across individual antidepressants. They recommend extra caution when discontinuing paroxetine, duloxetine, venlafaxine and desvenlafaxine, which showed the strongest disproportionate reporting of withdrawal. Particular

attention should also be paid to males, adolescents and younger adults, people taking antidepressants with other psychotropic drugs, and those on antidepressants for greater than 2 years. These subgroups may experience longer and more severe antidepressant-related withdrawal (Gastaldon et al., 2022).

Another under-recognised and under-reported adverse effect of antidepressants is post-SSRI sexual dysfunction (PSSD). This condition, characterised by genital numbness, pleasureless or weak orgasm, loss of libido, failure to become aroused and erectile dysfunction can persist long after the agent is discontinued (Horowitz & Taylor, 2024). Healy and Mangin (2024) recognise that people reporting PSSD have been ridiculed or have had their symptoms attributed to a mental health condition. Invalidating responses are based on the notion that the drug is no longer in the individual's system and therefore any ongoing dysfunction must be psychological. However PSSD has now been formally acknowledged by the European Medicines Agency, Health Canada, and the Australian Therapeutic Goods Administration. The prevalence and severity of PSSD though remains unclear due to lack of reporting, perhaps out of embarrassment, and questions asked by clinicians not sufficiently specific (Healy & Mangin, 2024).

Finally, Horowitz and Taylor (2024) raise concerns that the long-term use of antidepressants can induce tardive dysphoria. This is based on the proposition that a process of tolerance to antidepressants develops involving serotonin receptor desensitisation. This results in the opposite effect produced by the drug to that experienced when prescribed initially, indicating that long term use could be associated with poorer outcomes. The widely publicised umbrella review published by Moncrieff et al. (2022) reported "some evidence" that long-term antidepressant use reduces serotonin concentration.

Antipsychotics

The potency of antipsychotics is directly proportional to their ability to block dopamine receptors in the brain. They vary greatly in their selectivity for dopamine and have varying effects on acetylcholine, noradrenaline, histamine and serotonin pathways (Moncrieff, 2020; Taylor et al., 2021). Most drugs used in healthcare are presumed to work according to a 'disease-centred' model of drug action, whereby they act on the biological processes assumed to underpin symptoms. However, given there is no identified disease process associated with any psychiatric condition, an alternative 'drug centred' model proposes that psychotropic drugs change mental states and behaviour through the modification of normal brain functions (Read & Moncrieff, 2022). Antipsychotic drugs therefore act as a 'chemical straitjacket' by suppressing mental and physical activity (Moncrieff, 2020).

Antipsychotic drugs are considered effective for many people in reducing psychotic symptoms and are therefore recommended as first-line treatment (Leucht et al., 2013).

Maintenance antipsychotic treatment for at least 1–2 years is then recommended to prevent 'relapse'. However, it is estimated that antipsychotics are not effective for around 23% of people, and antipsychotics also cause harms such as unpleasant motor and metabolic effects, arrhythmias, sedation, cognitive impairment, emotional blunting, and amotivation (Allott et al., 2024). Six weeks of treatment with antipsychotics such as olanzapine and clozapine for example, is associated with an increase in body weight by approximately 3 kg (6.6 pounds). This predisposes people to a considerable risk of developing cardiovascular disease (Pillinger et al., 2020). Antipsychotics produce akathisia in a substantial percentage of recipients, which is known risk factor for suicide and violence (Whitaker, 2020). A survey of 832 people taking antipsychotics reported that 58% experienced 'suicidality' as an adverse effect of the drug, with 21% reporting it as 'severe' (Read & Williams, 2019). There is also evidence that antipsychotics are responsible for changes in brain structure (Hashimoto et al., 2018; Moncrieff & Leo, 2010; Tang et al., 2024). For example, Voineskos et al. (2020) found that exposure to olanzapine compared with placebo was associated with considerable decreases in cortical thickness in the left and right hemisphere.

The prevailing view in mental health services is that people should be maintained on antipsychotics long term. However, Harrow et al. (2017) argue that no strong evidence exists for treatment on antipsychotic drugs beyond 3 years. In their study, participants with 'schizophrenia' not on antipsychotics after the first 2 years have better outcomes than participants prescribed antipsychotics long term. Regardless of diagnosis, after the second year, the absence of antipsychotics was associated with a higher probability of recovery and lower probability of rehospitalization at subsequent follow-ups after adjusting for confounders (Harrow et al., 2022). Similarly, Bergström et al. (2020) conclude that moderate and high cumulative exposure to antipsychotics within the first 5 years from first episode psychosis was consistently associated with a higher risk of adverse outcomes during the 19-year follow-up, as compared to low or zero exposure.

In their meta-analysis comparing antipsychotic maintenance to discontinuation, Schlier et al. (2023) found that whilst short-term follow-ups (<2 years) on social functioning did not yield significant differences between maintenance and discontinuation, middle- and long-term follow-ups significantly favoured discontinuation. These authors identify multiple shortcomings in the design of RCTs, particularly short-term studies whereby abrupt switch to placebo increased incidents of relapse making maintenance look more favourable. Schlier et al. (2023) identify a need for high-quality longer-term studies to examine the effect of maintenance versus discontinuation or dose reduction on functional recovery before definitive conclusions can be drawn.

Moncrieff and Steingard (2019) also acknowledge that research on antipsychotic effectiveness is limited to short-term RCTs and flawed observational studies. They

recognise a need for long-term follow-up of randomised cohorts to determine the full risks and benefits of continuous antipsychotic treatment compared with other treatment strategies over the long-term.

Whitaker (2020) investigated claims that antipsychotics reduce mortality in patients diagnosed with ‘schizophrenia and other psychotic disorders’. The conclusions drawn from this review however found that people living with mental illness, a large percentage of whom are prescribed antipsychotics, die from physical conditions at rates two to five times that of the general population. The excess mortality is often attributed to the (yet to be discovered) ‘disease’ or blamed on unhealthy behaviours, as opposed to the adverse effects of antipsychotics. Yet antipsychotics elevate mortality in non-psychiatric patients and that non-psychiatric patients who take antipsychotics are at greater risk of dying than psychiatric patients who do not take them (Whitaker, 2020).

Relapse or withdrawal?

It is common for people prescribed antipsychotics to stop taking them, typically due to unpleasant effects (Read & Williams, 2019). When an individual experiences adverse effects from discontinuation their symptoms are usually labelled as ‘relapse’. However an alternative argument is made for this reaction being a form of drug withdrawal (Horowitz, Jauhar, et al., 2021; Horowitz, Murray, et al., 2021; Whitaker, 2020).

This is demonstrated by the occurrence of psychotic symptoms in people without a psychotic condition who abruptly stop taking antipsychotics prescribed to treat same ailments, such as nausea or lactation difficulties (Horowitz, Murray, et al., 2021). The severity of psychosis following clozapine withdrawal has also been noted to exceed that of the presenting condition before clozapine treatment (Horowitz, Jauhar, et al., 2021). Psychotic symptoms are often attributed to a ‘return of the disease’, however individuals report psychotic symptoms of a different kind, often more severe than their pre-drug psychosis. Symptoms include insomnia, restlessness, nausea, mood disturbances, severe psychosis, and newly emergent akathisia that may persist for long periods of time (Whitaker, 2020).

These symptoms can be grouped into somatic symptoms (nausea, sweating), motor symptoms, and psychological symptoms (including psychosis) and are attributed to the abrupt removal of the antagonistic cholinergic, adrenergic, serotonergic, and histaminergic effects of antipsychotics. The cholinergic rebound can also be characterised by agitation, fear, and hallucinations, which may be mistaken for relapse (Horowitz, Jauhar, et al., 2021).

Stimulants

“Attention Deficit Hyperactivity Disorder” (ADHD) is another psychiatric label diagnosed in increasing numbers and often treated with stimulant drugs. It is a vaguely defined condition with each of the 18 symptoms listed in the DSM-5 diagnostic criteria for ADHD beginning with

the subjective descriptor ‘often’ or ‘is often’ (Freedman & Honkasilta, 2017). The rise in ADHD has occurred despite there being no identifiable organic pathology, with youngest children in a school year, and children from socio-economic disadvantage identified as strong predictors of a diagnosis (Batstra et al., 2022; Schleim, 2022). Honkasilta and Koutsoklenis (2022) label the assertion that ADHD is a neurodevelopmental condition as scientific deceit. Neuroimaging studies used to justify ADHD as a ‘brain disease’ report small effect sizes and reveal that many classified with ADHD do not have smaller brains or brain parts than average, while the search to identify specific genes has been disappointing (Te Meerman et al., 2022).

Psychostimulants such as methylphenidate and amphetamines, and noradrenaline reuptake inhibitors are routinely prescribed to children and others diagnosed with ADHD. However, despite considerable professional pressure, the World Health Organization (2021) Expert Committee on Selection and Use of Essential Medicines ruled against methylphenidate being listed as an essential medicine for ADHD.

The expert review surmised that evidence for efficacy of methylphenidate is of low quality, there are numerous risks associated with methylphenidate, including cardiovascular risks, a worsening of psychiatric symptoms, and suppression of growth. There is an absence of sufficient data on long-term outcomes and there is a risk of misuse and dependence.

The cardiovascular risks associated with ADHD drugs were confirmed in a recent study conducted by Zhang et al. (2024). This large, nested case-control study found an increased risk of incidence of cardiovascular disease (especially hypertension and arterial disease) associated with ADHD drugs, with the risk rising following long-term use. This association was statistically significant for children, youth and adults, as well as for females and males.

Another case-control study published by Moran et al. (2024) identified increased odds of psychosis or mania with prescription amphetamine use, where increasing dose levels of prescription amphetamines were associated with higher risk. Prolonged ADHD drug use at higher doses is also significantly associated with smaller hippocampus volumes in specific subregions (Fotopoulos et al., 2021).

Mood stabilisers

Moncrief (2020) reports that the label of ‘mood stabiliser’ emerged in the 1990s when the pharmaceutical company that produced sodium valproate began marketing it for ‘bipolar disorder’. However, there is no evidence that mood stabilisers normalise emotional responses or stabilise mood. Any drug with sedating properties is likely to have a therapeutic effect with a condition characterised by heightened arousal and activity such as mania (Moncrief, 2020).

Lithium (traditionally referred to as a “gold standard” treatment) is a toxic alkali metal and is often claimed to have special anti-suicide qualities, though the evidence for

this is weak (Gøtzsche, 2024; Moncrief, 2020). Nabi et al. (2022) conclude that evidence from randomised trials does not support the assertion that lithium prevents suicide or suicidal behaviour, and the largest trial of lithium for suicide prevention was terminated early due to lack of effect (Katz et al., 2022).

Suicide is a complex phenomenon that needs to be understood in terms of individual, social and cultural factors. To suggest that a single chemical such as lithium can counter the outcome of this complex interaction is unrealistic. If lithium does prevent suicide, a preventive effect on suicide attempts should also be seen, and it is not (Moncrieff & Plöderl, 2024). Baldessarini and Tondo (2022) have acknowledged the uncertainty of lithium's anti-suicidal properties and that recruiting participants to suicide prevention trials may be made difficult by a prevailing belief that the question of anti-suicidal effects of lithium is already settled, 'which it certainly is not.'

Lithium can cause hand tremor, slurred speech, lethargy, diarrhoea, diminished reaction times, slow thinking, and blow_up creativity while up to 50% of women on long term treatment require thyroid hormones (Moncrief, 2020). People treated with lithium are also at increased risk of chronic kidney disease after long-term exposure. Whereas some people tolerate lithium for up to several decades without developing kidney disease, many do not (Fransson et al., 2022).

Drugs that are usually used to treat epilepsy, such as sodium valproate are essentially sedative drugs that suppress nervous activity in different ways. While serious complications are uncommon, sodium valproate can cause nausea, lethargy, sedation, hair loss, weight gain and polycystic ovaries (Moncrief, 2020).

Decreasing and discontinuing psychotropic drugs

Gøtzsche and Demasi (2024) explain that the incidence and severity of antidepressant drug withdrawal depends on how the dose is tapered, as symptoms can occur even following small dose reductions. After long-term exposure to a drug, physiological adaptations take time to return to normal. This leaves the affected receptors in temporary disequilibrium following dose reduction, from which withdrawal symptoms arise. A linear tapering regime or halving the dose each time corresponds to increasingly larger reductions in receptor occupancy and heightens the risk of withdrawal symptoms. Hyperbolic tapering is therefore required, consisting of numerous dose reductions below the lowest manufactured dose. Hyperbolic tapering is painstaking, involving dividing the smallest dose into smaller units, for example by using a nail file, dispersing the drug in water, counting beads from capsules, or using tapering strips (Gøtzsche & Demasi, 2024).

It has been identified that a lack of knowledge, awareness and guidance from clinicians on psychotropic drug withdrawal, and how to safely discontinue these drugs, has led to a burgeoning number of social media support groups filling the gap left by health services (Coe et al.,

2024; Groot & van Os, 2021). In an international survey of people who use online support groups to taper and cease their antidepressants ($n = 967$), respondents reported widely varied (and predominantly negative and unhelpful) responses and guidance from their prescriber when discussing discontinuation. Many prescribers were unaware or denied that stopping antidepressants could cause withdrawal. Over 70% of respondents felt the advice received on how to stop antidepressants was unhelpful, mostly because the recommended tapering regime was too quick. Most prescribers recommended tapering over 1 month or less (Read et al., 2023).

To address this deficit in understanding, knowledge and service delivery Horowitz and Taylor (2024) have published a detailed textbook for deprescribing antidepressants, benzodiazepines, gabapentinoids and Z-drugs. For example, they provide suggested taper schedules for 25 different antidepressants, with the central message to go slowly, at a rate at which the individual can tolerate, and proceeding more cautiously for the last few milligrams which are typically the hardest to stop. Minute doses are likely to be needed for most long-term users in order to stop safely (Horowitz & Taylor, 2024).

Studies undertaken by Groot and van Os (2021) have established a 72% success rate using tapering strips for discontinuing antidepressants. These consist of antidepressants, packaged in a roll or strip of small daily pouches. Each pouch is numbered and has the same or slightly lower dose than the one before it. However tapering strips are only available in certain locations and are prepared by compounding pharmacies and therefore expensive. Groot and van Os (2021) state that the preferred approach is prevention, using psychotropic drugs more conservatively, and taking into account that some drugs such as paroxetine and venlafaxine are associated with a higher likelihood of withdrawal than others.

Wallis et al. (2024) present an interesting comparison of antidepressant use between the Netherlands and Australia (two countries with comparable health services, quality of life and happiness). Australia has a much higher rate of antidepressant drug prescribing. The authors attribute the difference in antidepressant use to a number of factors. In the Netherlands there is funding for mental health nurses (MHN) in general practice, free psychological therapy, and detailed clinical guidance on tapering and stopping antidepressant use, complemented by resources such as tapering protocols and strips. There is also tighter regulation of pharmaceutical industry influence on prescribers. Moreover, in the Netherlands, after completing their generalist nursing program, individuals pursuing a mental health specialisation undergo extensive post-graduate training, in contrast to many countries where MHN specialisation is typically shorter.

With regard to antipsychotic drugs, Horowitz, Jauhar, et al. (2021) propose that dopaminergic hypersensitivity contributes to early relapse into psychosis after antipsychotic drug cessation. The concept of neuroadaptations that take months or years to resolve

during tapering and following discontinuation means that deprescribing guidance on how best to reduce and potentially cease antipsychotic drugs are urgently needed. Similar to antidepressants, they advocate for a slow and gradual reduction, over months or even years, with slower reductions at lower doses to afford time for neuroadaptations to occur, thereby limiting the risks of withdrawal symptoms and relapse. Depot preparations of antipsychotics provide a useful option as their extended half-lives represent a form of “in-built” tapering (Horowitz, Jauhar, et al., 2021).

Cooper et al. (2021) provide pragmatic suggestions on antipsychotic reduction and discontinuation. People may require more intensive monitoring and support during antipsychotic drug reduction and for a time after they have stopped. Importantly, individualised approaches are needed. Reducing to the lowest possible dose may be more plausible than stopping, and some people may find intermittent antipsychotic drug use helpful. Information about possible withdrawal effects of antipsychotics, such as insomnia, and the management of anxiety through psychological support, self-management strategies and short-term medication (such as benzodiazepines) is likely to be important. Some people will need support in processing emotions that were previously blunted by antipsychotics (Cooper et al., 2021).

Implications for mental health nursing practice The WHO and UN (2023) affirm that people should be able to exercise their right to give free and informed consent to accept or reject treatment in mental health systems. Health professionals should inform people about their right to discontinue treatment and to receive support in this. Support should be provided to help people safely withdraw from treatment with drugs (WHO & UN, 2023). The International Council of Nurses (2021) code of ethics states that nurses must ensure people receive accurate, sufficient and timely information on which to base consent for care and treatment. MHNs therefore need to be equipped with contemporary knowledge of psychotropic drug effectiveness, adverse effects, and withdrawal effects in order to inform and support people in safely reducing or ceasing their psychotropic drugs if they decide to do so.

Wand (2024) proposes that MHNs are well placed to establish and lead psychotropic drug deprescribing and discontinuation services. A collaborative, person-centered (rather than diagnosis driven) approach is indicated which respects the importance of informed and supported decision-making. Incorporating principles of Lifestyle Medicine (Fallows et al., 2023), MHNs could coordinate care with colleagues from medicine, psychology, lived experience, dietetics and exercise science to facilitate individually tailored programs that support people in gradually decreasing and potentially discontinuing their psychotropic drugs based on personal preferences (Wand, 2024).

Conclusion

This critique has highlighted that the justification for the effectiveness of psychotropic drugs is based mainly on short-term clinical trials that employ blunt instruments to assess their worth. Given that people are routinely informed they need to take these agents long term, robust longitudinal studies are required that investigate a range of aspects such as quality of life, productivity, morbidity, mortality and the human experience of taking psychotropic drugs. With the knowledge that there is no clear or direct neuro-biogenic cause for mental illness or identifiable disease process, the indication is that psychotropic drugs provide no more than a crude form of chemical restraint. While this may have appeal for blunting mental distress, the long-term trade-off for the adverse effect burden and consequences when trying to discontinue these agents needs to be considered by clinicians and openly discussed with people in their care. If psychotropic drugs are prescribed then the overriding principle is that they should be used conservatively, at the lowest dose and for the shortest time possible.

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